

Discharge Summary

****Patient:**** John Doe | ****DOB:**** 1958-03-12

History of Present Illness

The patient is a 54-year-old male with a history of CHF who presented to the hospital with acute decompensated heart failure. No prior admission note is available for this record.

Hospital Course

The patient was admitted for management of acute-on-chronic heart failure. He was started on intravenous diuretics with good initial urine output. His weight decreased by 3 kg over the first 48 hours. Lisinopril was continued at 10 mg daily. Laboratory studies showed improvement in BNP from 1200 to 450 pg/mL and normalization of renal function. He remained hemodynamically stable with no evidence of arrhythmias. By hospital day 3, he was tolerating oral intake, ambulating independently, and had no orthopnea. He was deemed stable for discharge on oral medications.

Discharge Medications

- Lisinopril 10 mg orally once daily (continued from admission, no dose change)

Follow-up

- Follow up with primary care physician within 7 days.
- Follow up with cardiology in 2 weeks.
- Return to the emergency department for worsening shortness of breath, chest pain, dizziness, or significant weight gain (>2 kg in 24 hours).
 - Allergies: Penicillin (reported).
 - Contact: Hospitalist service (555-0100) for non-urgent questions.